Medical Record Summary — Chronological (AI-readable)

Patient profile

Field	Value
Name	M. Mohsin Chowdhury
Sex / DOB	Male / 11 Jul 1967 (age 59 as of Oct 2026)
Hospital IDs	Apollo Imperial Hospital (IHL), Chattogram: MRN IHL1037895 · Apollo Bangalore UHID AHB.0000752138 · Popular Diagnostic UHID 150361302 / 151042787
Anthropometry (12 Feb 2025)	Weight 80.6 kg, height 167 cm, BMI 28.9
Main labs used	Labaid Specialized Hospital Chattogram, Apollo Imperial Hospital (IHL) Chattogram, Popular Diagnostic Centre Chattogram
Main physicians	Dr. Mohammad Mustafizur Rahman (Internal Medicine, IHL); Dr. A.T.M. Mohiuddin Chowdhury (Gastroenterology, IHL); Dr. Mohammad Majibul Haque (Internal Medicine, IHL, Sep 2026 admission)

Long-standing conditions (background)

- **Idiopathic hypersomnia** since 1989; **parasomnia** since Jun–Jul 2024. Treated with modafinil/armodafinil + methylphenidate (Ritalin/Retanil).
- **Morbid obesity → laparoscopic mini gastric bypass / one-anastomosis gastric bypass (OAGB/MGB)**, 4 Jul 2017 (Apollo Bangalore). Gastrojejunal anastomosis, 155 cm limb. (In 2026 endoscopy notes this is called “partial gastrectomy / post-gastrojejunostomy” anatomy.)
- **Hypertension**: listed in 2017; IHL notes say since 2022. Nebivolol 5 mg; later telmisartan / amlodipine.
- **Dyslipidaemia**: statin intolerant, also did not tolerate ciprofibrate. Started inclisiran (Leqvio) in Mar 2025.
- **Restless legs syndrome**: pramipexole (Parixol) 0.088 mg at night.
- Idiopathic muscle cramps: pregabalin 165 mg BD, stopped Sep 2024. Mirtazapine also stopped.
- Other regular meds (Feb 2025): escitalopram 10 mg OM, midazolam 7.5 mg at night (sleep), vitamin B12.
- Non-smoker. Not diabetic.

One-paragraph synopsis

A man with prior OAGB bariatric surgery (2017) developed fatigue, low libido and a “feverish” feeling in late 2024. In Feb 2025 he was found to have **severe iron-deficiency anaemia** (Hb ~10, MCV 67, ferritin 4.55), vitamin D insufficiency, high LDL (189), and serology positive for **enteric (typhoid) and rickettsial fever**. He was admitted on 14–15 Feb 2025 and treated with ceftriaxone, doxycycline and then levofloxacin. That course was followed by **antibiotic-associated (pseudomembranous) colitis**, treated with vancomycin. Endoscopy on 4 Mar 2025 showed **erosions at the gastro-enterostomy stoma**. Colonoscopy on 20 Mar 2025 found a 15 mm splenic-flexure lesion/polyp, removed by hot biopsy; histology showed **non-specific chronic inflammation, no malignancy**. Stool H. pylori antigen and occult blood were negative at that time. **IV iron (ferric carboxymaltose/Maltofer)** fully corrected the anaemia (Hb 13.8–15.1, ferritin 107–239) from Apr 2025 to Mar 2026, and inclisiran brought LDL down to 56–78. In **Jun 2026** endoscopy showed a **chronic ulcer at the bypass stoma, pangastritis, and H. pylori positive (RUT +++)**; ultrasound showed grade-I fatty liver and new **gallstones**. On **8 Sep 2026** he had a **presyncopal fall with head trauma**. He was found to have **melaena with an acute drop in Hb to 8.2**,

from a **chronic ulcer just below the anastomosis (H. pylori RUT ++)**, plus **bile reflux** and a **hiatus hernia (Hill grade IV)**. Brain MRI showed mild atrophy and grade-I microvascular change. EEG, echo and carotid Doppler were normal. He was treated with esomeprazole, sucralfate, tranexamic acid, and an H. pylori regimen (amoxicillin-clavulanate + levofloxacin). By **13 Oct 2026** Hb had recovered to 12.1, but ferritin was low again on 29 Sep 2026 (11.2): **iron deficiency has recurred**.

Chronological timeline

04 Jul 2017 — Bariatric surgery admission (Apollo Hospitals, Bangalore) [OAGB Surgery Discharge Report 7 July 2017.pdf]

- Admitted 03 Jul, discharged 07 Jul 2017. Age 49.
- Diagnoses: morbid obesity, hypertension, hypersomnia, dyslipidaemia. Not diabetic, no CAD, no asthma.
- **Procedure: laparoscopic mini gastric bypass (LAP MGB) under GA, 04 Jul 2017.** Intra-op: enlarged liver, small gastric pouch, gastrojejunal anastomosis at 155 cm. Uneventful post-op; gastrografin study normal.
- Discharge meds: pantoprazole 40 mg BD, multivitamin (Zevit), nebivolol 5 mg, Bizoran (amlodipine/olmesartan) 20/5, armodafinil 150 mg, Ritalin 10 mg BD, niacin + aspirin 500/75, vitamin D3 60 000 IU weekly ×8 weeks, calcium (CCM), probiotic (Andoxin) ×3 months.

Late 2023 – Jan 2025 — Symptom onset [Summary 26 March 2025.docx]

- About 1.5 years of weakness, lethargy, loss of interest, and **libido falling to zero** despite PDE5 inhibitors.
- Since **Dec 2024**: feels feverish without measured fever.
- Pica: has been eating raw rice for 6–7 months (sign of iron deficiency), and has a perverted sense of taste.
- Took a 5-day azithromycin course in early Feb 2025.

08 Feb 2025 — First labs (Labaid; ref. Dr. Johor Dutta)

Test	Result	Ref
Hb / MCV / MCH / RDW-CV (from Rx note)	**10.0 L / 67.4 L / 20.8 L / 19.1 H**; WBC 7 600; Plt 360 000; ESR 28	—
CRP	**6.4 H** mg/L	<5
ALT / AST	34 / 29 U/L	≤41 / ≤37
Creatinine / eGFR	**1.28** mg/dL / 66	0.7–1.3
CK (CPK)	**220 H** U/L	24–190
LDL	**189 (high)** mg/dL	<100
Triglycerides	**164 H** mg/dL	<150
TSH	1.19 µIU/mL	0.3–4.5
Total testosterone	3.66 ng/mL (the prescription records it as 3.99)	1.71–7.89
Vitamin D 25-OH	**18.1 (insufficient)** ng/mL	30–100

12 Feb 2025 — OPD, Internal Medicine, IHL (Dr. Mustafizur Rahman) [Apollo Imperial Hospital Prescriptions Feb-Mar25.pdf p5–6]

- Vitals: T 98.4 °F, P 76, BP 130/80, SpO_2 99%.
- Complaint: intermittent low-grade fever with body ache ×1 month.
- Provisional diagnosis: **microcytic hypochromic anaemia; PUO** (pyrexia of unknown origin); vitamin D deficiency.
- Rx: ciprofibrate 100 mg, vitamin D 200 000 IU ×2 doses, albendazole, paracetamol.
- Ordered: iron studies, blood culture, typhoid serology, triple antigen, urine RME, CEA, CA 19-9, OGTT, calcium, chest X-ray, abdominal ultrasound.

13 Feb 2025 — Work-up (Labaid + Popular Diagnostic)

Test	Result	Ref
Ferritin	**4.55 L** ng/mL	25–350
Serum iron	**28 L** µg/dL	65–175
TIBC	411 µg/dL	250–450
Salmonella Typhi IgG / IgM (ICT)	**Positive / Positive**	—
Triple antigen	Rickettsia OX2, OXK, OX19 **1:160 (significant, >1:80)**; Brucella 1:80 (not significant); Widal TO/TH/AO/AH/BO/BH all 1:80 (not significant, cut-off ≥1:160)	—
Blood culture	No growth	—
CEA	2.66 ng/mL (negative)	<5.09
CA 19-9	2.54 U/mL (negative)	<39
Calcium	8.7 mg/dL	8.4–10.4
Urine RME	Albumin trace; no RBC; pus cells 1–2	—
Vitamin B12 (Popular)	527 pg/mL	179–1162
Chest X-ray PA (Labaid)	Normal	—
USG whole abdomen + PVR (Popular)	Liver, GB, pancreas, spleen (8.1 cm) normal. **Right renal cortical cyst 23×19 mm, thin septa, benign.** PVR 20 ml. Prostate 17.6 ml, normal.	—

14–15 Feb 2025 — Admission, IHL Internal Medicine [Apollo Imperial Hospital Prescriptions Feb-Mar25.pdf p3–4]

- **Diagnosis: enteric fever with rickettsial fever, with HTN.** BP 120/80, P 75, afebrile on exam, CBG 5 mmol/L.
- Discharge Rx: **ceftriaxone 2 g IV BD (14–27 Feb)**, later switched by hand to cefuroxime (Roxim) 400 BD; **doxycycline 100 mg BD (14–27 Feb)**; pantoprazole 20 mg BD ×15 days; ciprofibrate 100 mg; modafinil ¼ BD; Retanil 10 mg ¼ BD; pramipexole 0.088 mg; midazolam 7.5 mg PRN; nebivolol 5 mg; escitalopram 10 mg; paracetamol PRN; albendazole.
- Advice: blood culture, endoscopy + colonoscopy, IV Maltofer 1 g then 500 mg.
- Patient's own summary: levofloxacin 500 mg OD 22–28 Feb was also given.
- Repeat CBC 16 Feb 2025: Hb 9.9, MCV 64.6, MCH 20.7, RDW 20.6, Plt 396 000.

02 Mar 2025 — Hb electrophoresis (IHL) [Capillary Hb Electrophoresis.pdf]

- Hb 10.3 g/dL; **HbA 98.1%, HbA2 1.9% (low)**. Comment: low HbA2, consistent with iron deficiency; no β-thalassaemia trait pattern.
- **IV Maltofer 1 g given 02 Mar 2025** (patient summary).
- 02 Mar complaint: generalized weakness after the fever, and loose stools while on antibiotics.

04 Mar 2025 — CBC + upper GI endoscopy (Labaid)

- CBC [CBC 04Mar2025.pdf]: **Hb 9.9 L, Hct 31.5 L, MCV 69.1 L, MCH 21.8 L, RDW-CV 21.1 H**; WBC 7.8; Plt 350; ESR 15.
- **Upper GI endoscopy** [Upper GI Tract Endoscopy 4 Mar25.pdf]: oesophagus normal. Resected stomach with **erosions at the margin of the gastro-enterostomy stoma** and the adjacent stomach. Afferent and efferent loops normal. No biopsy taken.

08 Mar 2025 — OPD follow-up, IHL [Apollo Imperial Hospital Prescription 8Mar25.pdf; also p1–2 of the Feb-Mar file]

- Diagnoses: **vitamin D deficiency with iron-deficiency anaemia; antibiotic-associated pseudomembranous colitis; gastric erosion.** PUO still under investigation.
- Rx: rosuvastatin 5 mg at night, CoQ10 100 mg, **folic acid 5 mg ×3 months**, **vonoprazan 20 mg BD ×2 weeks → OD ×6 weeks → PRN**, vitamin D 200 000 IU ×2, probiotic ×14 days, **vancomycin 250 mg TDS ×7 days**, Maltofer 1 g + 500 mg IV, albendazole, paracetamol, ORS, **inclisiran (Leqvio) SC on day 1, then at 3 and 6 months**.
- Planned: CBC/ESR and CPK in 6 weeks; total colonoscopy; endoscopy with RUT for H. pylori.
- Patient summary: **Maltofer 500 mg on 10 Mar 2025; Leqvio given 14 Mar 2025** (next doses due 14 Jun and 14 Dec 2025).

19 Mar 2025 — Stool tests (IHL)

- **H. pylori stool antigen: negative** [H. Pylori .pdf]. Note: this was done while on/after vonoprazan, so it may be a false negative.
- **Stool occult blood: negative** [Stool Occult Blood.pdf].

20 Mar 2025 — Colonoscopy + polypectomy (IHL, Dr. A.T.M. Mohiuddin Chowdhury) [Colonoscopy and Biopsy Report 20 March 2025.pdf; Biopsy .pdf]

- Examined up to the terminal ileum. **Elevated growing-type lesion / polyp, about 15×15 mm, at the splenic flexure**, removed with hot biopsy forceps; haemostasis achieved. Few scattered erosions in the transverse and descending colon. Rest normal.
- **Histopathology (reported 24 Mar 2025): non-specific chronic inflammation, no evidence of malignancy.**

26 Mar 2025 — Patient-written status [Summary 26 March 2025.docx]

- Still weak and feels feverish, without measured fever.

17 Apr 2025 — Post-IV-iron check (Labaid)

- CBC: **Hb 13.8 (normalised)**, MCV 86.7, **RDW-CV 32.0 H / RDW-SD 101.3 H** (mixed old and new red-cell populations after iron), ESR 02.
- Ferritin **107** ng/mL; serum iron 61 (slightly low); creatinine 0.75, eGFR 120.

14 Jun 2025 — CBC (Labaid)

- **Hb 15.1**, MCV 91.4, RDW-CV 15.5 H, ESR 05. Normal.

27 Jul 2025 — Labaid

- **LDL 56** mg/dL (from 189; on inclisiran ± rosuvastatin), TG 125.
- Ferritin 207. **ANA 20.8 AU/mL, negative.**

04 Sep 2025 — Labaid

- CBC: Hb 14.7, WBC 10.09 (upper normal), neutrophils 65%, RDW-CV 15.1 H, ESR 02.
- Ferritin 193.
- **Salmonella Typhi IgG negative / IgM positive** (ICT; reason for the test not documented).

02 Nov 2025 — Labaid panel

Test	Result	Ref
Ferritin	238.75	25–350
Serum iron	126	65–175
CRP	1.4	<5
LFT	Bilirubin 0.70, AST 24, ALT 27, ALP 49	normal
Calcium / Mg / K	9.1 / 1.9 / 3.70	normal
Uric acid	5.4	3.5–7.2
Free testosterone	16.93 pg/mL	9.57–40.6 (age 40–59)
Prolactin	**2.19 L** ng/mL	3.45–17.42
Blood culture	Report page has no result printed	—

27 Feb – 16 Mar 2026 — Labaid

- 27 Feb: CBC **Hb 13.8**, MCV 95.9, MCH 32.3 H, RDW-CV 15.4 H, ESR 10. Creatinine 0.97, eGFR 92.
- 02 Mar: Ferritin 148.10. LFT: bilirubin 0.4, AST 19, ALT 40, ALP 99.
- 16 Mar: LDL 78, TG 95. **Total testosterone 5.51** ng/mL (normal). **Free testosterone 12.22** pg/mL (low-normal for age).

07 Jun 2026 — Upper GI endoscopy (IHL) + abdominal ultrasound (Popular)

- **Endoscopy** [Endoscopy Report 7 June 2026.pdf]: post-gastrojejunostomy bypass status. Oesophagus normal, LES lax. **Chronic white-base ulcer at the bypass stoma; inflamed gastric mucosa with erythema and erosions throughout (pangastritis).** Loops intact. Biopsy taken for RUT and histopathology; **RUT for H. pylori: positive +++**. No histopathology report for this biopsy is in the folder.
- **USG whole abdomen + PVR** [Ultrasonogram of Whole Abdomen 7 June 2026 .pdf]: **fatty liver grade I** (raised hepatorenal contrast); **cholelithiasis** (multiple stones 5.4–5.7 mm), CBD not dilated; right renal cortical cyst **27×19 mm** (was 23×19 in Feb 2025), benign; PVR 12 ml; prostate 16.8 ml.

08–10 Sep 2026 — Emergency admission, IHL Internal Medicine (Dr. Majibul Haque) [IHL Discharge Summary 10 Sept 2026.pdf]

- **Presentation (08 Sep, 00:43):** presyncopal attack, then a fall with **blunt trauma to the occiput**. GCS 15/15, BP 100/70, P 92, SpO_2 100%, T 98 °F, CBG 7.3 mmol/L, dehydration ++, bilateral extensor plantar responses, power 5/5.
- **Discharge diagnosis: duodenal ulcer with melaena**, with HTN, history of bariatric surgery, dyslipidaemia, and idiopathic hypersomnia with parasomnia.
- **Labs:**
  - 08 Sep CMP: glucose 6.1 mmol/L; **total protein 5.3 L**; albumin 3.6; **calcium 8.1 L**; Na 137; K 3.9; Cl 108; **HCO_3 21 L**; BUN 23 (upper limit); creatinine 0.89; eGFR 87; LFT normal.
  - 09 Sep CBC: **Hb 8.7 L, Hct 24.1, RBC 2.74**, MCV 88, Plt 200, WBC 8.13.
  - 10 Sep CBC: **Hb 8.2 L, Hct 23.1, RBC 2.62**, MCV 88.2, Plt 220, WBC 7.88.
  - 09 Sep coagulation: PT 11.9 s, INR 1.04, APTT 28.2 s, fibrinogen 2.2 g/L, D-dimer 0.13 (all normal).
  - 09 Sep stool: **blackish, offensive**, semi-solid; RBC 0–2/HPF; **occult blood positive**.
- **Imaging / tests:**
  - **MRI brain, epilepsy protocol (08 Sep)**: mild generalized cerebral atrophy; **bilateral chronic microvascular white-matter ischaemic change (grade I)**; hippocampi normal; no acute infarct, haemorrhage, mass or dysplasia.
  - **Bedside EEG (09 Sep)**: normal awake EEG, no epileptiform discharges. Excess bilateral frontal beta, suggesting an anxiety state. HR 90–96.
  - **Transthoracic echo (09 Sep)**: normal chambers and valves, **LVEF 61%**, TAPSE 20 mm, no RWMA, no effusion. TR Vmax 193 cm/s (PG 15 mmHg).
  - **Carotid + vertebral Doppler (09 Sep)**: normal. IMT 0.3–0.4 mm, no plaque.

    - **Upper GI endoscopy + RUT (09 Sep)** [Endoscopy Report 9 Sept 2026.pdf]: partial-gastrectomy status; **large amount of bile in the stomach**; **hiatus hernia**; duodenal bulb absent; **large chronic white-base ulcer just below the anastomosis margin**; **RUT H. pylori positive ++**. Impression: **chronic (duodenal/anastomotic) ulcer post gastrectomy; bile reflux disease; hiatus hernia, Hill grade IV.** ("Endoscopy 9 Sept 26 without Report.pdf" is the same study's images.)
- **Discharge Rx (10 Sep 2026):** amoxicillin-clavulanate 1 g TDS ×14 days + levofloxacin 500 mg OD ×14 days (H. pylori eradication); **esomeprazole 20 mg BD**; sucralfate 1 g/5 ml 2 tsp 6-hourly ×2 months; **tranexamic acid 500 mg TDS ×3 days**; domperidone 10 mg TDS ×3 months; folic acid 5 mg BD ×2 months; telmisartan 40 mg (handwritten: changed to amlodipine 10 mg at night); clonazepam 0.5 mg at night ×15 days; ispaghula; paracetamol PRN; povidone-iodine ointment to the scalp bruise. Handwritten: continue methylphenidate and modafinil. Follow up with Dr. Mohiuddin Chowdhury (GI) in 15 days.

29 Sep 2026 — Post-bleed check (Labaid)

- CBC: **Hb 9.9 L, Hct 30.5 L, RBC 3.48 L**, MCV 87.5, RDW-CV 15.6 H, WBC 5.51, Plt 369, ESR 12.
- **Ferritin 11.20 L** ng/mL (from 148 in Mar 2026): **iron stores depleted again.**
- Note: "CBC 11 Sept 2026.pdf" is byte-identical to "CBC 29 Sept 2026.pdf" (sample date 29/09/2026). There is no separate 11 Sep result.

13 Oct 2026 — Latest labs (Labaid)

- CBC: **Hb 12.1 L** (recovering), Hct 36.9 L, RBC 4.24, MCV 84.4, RDW-CV 16.3 H, WBC 5.32, **lymphocytes 48% H**, Plt 280, ESR 05.
- Calcium 9.0; total protein 7.6; albumin 3.7; bilirubin 0.40; AST 19; ALT 35; ALP 109; **creatinine 1.06, eGFR 82**.
- Phosphate is named in the file title, but no phosphate result appears in the report.
- **Folate 13.3 ng/mL** (ref 4.6–34.8) and **vitamin B12 690.39 pg/mL** (ref 192–827): both normal, so the anaemia is not due to folate or B12 deficiency. Folate was measured while on folic acid 5 mg BD. Sample taken 13 Oct 2026, reported 17 Oct 2026; referred by Dr. Surajit Roy Chowdhury (Psychiatry). [Folate and Vitamin B12 13 October 2026.pdf]

Trend tables

Haemoglobin & red cells

Date	Hb (g/dL)	MCV (fL)	RDW-CV (%)	Context
08 Feb 2025	10.0 L	67.4 L	19.1 H	Iron deficiency
16 Feb 2025	9.9 L	64.6 L	20.6 H	
02 Mar 2025	10.3 L	—	—	Maltofer 1 g given
04 Mar 2025	9.9 L	69.1 L	21.1 H	
17 Apr 2025	13.8	86.7	32.0 H	After IV iron
14 Jun 2025	15.1	91.4	15.5 H	
04 Sep 2025	14.7	91.7	15.1 H	
27 Feb 2026	13.8	95.9	15.4 H	
09 Sep 2026	8.7 L	88.0	14.2 H	Melaena / ulcer bleed
10 Sep 2026	8.2 L	88.2	14.3 H	Lowest value
29 Sep 2026	9.9 L	87.5	15.6 H	
13 Oct 2026	12.1 L	84.4	16.3 H	Recovering

Iron studies

Date	Ferritin (ng/mL)	Serum iron (µg/dL)	TIBC
13 Feb 2025	**4.55 L**	**28 L**	411
17 Apr 2025	107	61 L	—
27 Jul 2025	207	—	—
04 Sep 2025	193	—	—
02 Nov 2025	238.75	126	—
02 Mar 2026	148.10	—	—
29 Sep 2026	**11.20 L**	—	—

Lipids (statin-intolerant; inclisiran from 14 Mar 2025)

Date	LDL (mg/dL)	TG (mg/dL)
08 Feb 2025	**189**	**164 H**
27 Jul 2025	56	125
16 Mar 2026	78	95

Kidney function

Date	Creatinine (mg/dL)	eGFR
08 Feb 2025	1.28	66
17 Apr 2025	0.75	120
27 Feb 2026	0.97	92
08 Sep 2026	0.89	87
13 Oct 2026	1.06	82

Hormones

Date	Total T (ng/mL)	Free T (pg/mL)	Other
08 Feb 2025	3.66	—	TSH 1.19
02 Nov 2025	—	16.93	**Prolactin 2.19 L**
16 Mar 2026	5.51	12.22	

Vitamins

Date	Vitamin B12 (pg/mL)	Folate (ng/mL)	Vitamin D 25-OH (ng/mL)
08–13 Feb 2025	527	—	**18.1 (insufficient)**
13 Oct 2026	690.39	13.3	—

Inflammation / infection serology

Date	CRP (mg/L)	Typhoid ICT IgG/IgM	Other
08–13 Feb 2025	6.4 H	+/+	Rickettsia OX2/OXK/OX19 1:160; blood culture negative
04 Sep 2025	—	−/+	
02 Nov 2025	1.4	—	

GI findings over time

Date	Study	Key finding	H. pylori
04 Mar 2025	Upper GI endoscopy	Erosions at gastro-enterostomy stoma	Not tested
19 Mar 2025	Stool tests	Occult blood negative	Stool antigen negative (possibly suppressed by vonoprazan)
20 Mar 2025	Colonoscopy	15 mm splenic-flexure lesion removed (benign, chronic inflammation); scattered colonic erosions	—
07 Jun 2026	Upper GI endoscopy	Chronic stomal ulcer + pangastritis	**RUT +++**
09 Sep 2026	Upper GI endoscopy	Large chronic ulcer below anastomosis; bile reflux; hiatus hernia Hill IV; melaena	**RUT ++**

Active problem list (as of 13 Oct 2026)

1. **Chronic marginal (anastomotic) ulcer after OAGB, with an upper-GI bleed (melaena) in Sep 2026.** H. pylori positive in Jun and Sep 2026; eradication course started 10 Sep 2026. No eradication test result is on file yet.
2. **Recurrent iron-deficiency anaemia**: Hb 12.1 (recovering), ferritin 11.2 on 29 Sep 2026. Malabsorption after bypass plus GI blood loss is the likely mechanism. Folate (13.3) and B12 (690) were normal on 13 Oct 2026.
3. **Bile reflux disease; hiatus hernia (Hill grade IV).**
4. **Presyncope with fall and occipital head trauma (Sep 2026).** MRI: mild atrophy and grade-I small-vessel change. EEG, echo and carotids normal. Likely related to anaemia, volume loss and low BP (100/70).
5. **Cholelithiasis** (asymptomatic on report) and **grade-I fatty liver** (Jun 2026).
6. Benign right renal cortical cyst, slowly enlarging (23→27 mm, Feb 2025 → Jun 2026).
7. Hypertension (antihypertensive changed on discharge in Sep 2026).
8. Dyslipidaemia, controlled on inclisiran (LDL 78).
9. Idiopathic hypersomnia with parasomnia; restless legs syndrome; anxiety features on EEG.
10. Low prolactin (Nov 2025) and previously reported low libido; testosterone normal.
11. History (Feb 2025): enteric and rickettsial fever, treated; antibiotic-associated colitis, treated.

Gaps / data-quality notes for the reader

- No histopathology result for the Jun 2026 stoma-ulcer biopsy, and no H. pylori eradication test after the Sep 2026 regimen.
- The Nov 2025 blood culture report shows no organism or result line.
- The testosterone value for 8 Feb 2025 differs between the lab report (3.66) and the prescription (3.99).
- Duplicates: “CBC 11 Sept 2026” = “CBC 29 Sept 2026”; the two “COAGULATION PROFILE 9 Sept 2026” files are identical; “Biopsy .pdf” is the 20 Mar 2025 colonoscopy histopathology; “Testosterone and Free Testosterone 16 March 2026” duplicates “Free Testosterone and Total Testosterone 16 March 2026”; “Apollo Imperial Hospital Prescription 8Mar25” is the first two pages of the “Feb-Mar25” file.
- The echo file is named 8 Sept, but the report itself is dated 09 Sep 2026.
- Reference ranges differ between labs (Labaid, IHL, Popular), so compare each value with its own lab’s range.